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CFI - Care: Collaborative Flow of Intelligence for Care

Graduation Project Thesis

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Submission Date: 04 April, 2025

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Abstract

Healthcare today is characterized by fragmented experiences where patient medical information is often dispersed across various institutions and providers. This project proposes a comprehensive, patient-centric healthcare application designed to consolidate all medical reports into a single, secure, and unified platform. The system employs Role-Based Access Control (RBAC) to ensure privacy and controlled information sharing. To enhance clinical decision-making, the platform features a medical history visualization graph for tracking health trends and integrates a Retrieval-Augmented Generation (RAG)-enabled AI model to generate diagnostic reports and insights. By transforming isolated data into an interconnected ecosystem, this project aims to establish meaningful relationships between patients, caregivers, and doctors for personalized healthcare delivery.

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Chapter 1

Introduction

Today, patients often face fragmented healthcare experiences, with their medical information dispersed across multiple institutions and providers. This project proposes a patient-centric healthcare application designed to consolidate all medical reports within a single, secure, and unified platform.

The system empowers patients to manage their health through continuous data collection, secure communication, and controlled information sharing using Role-Based Access Control (RBAC) to ensure privacy, consent, and data integrity. Additionally, by integrating features such as medical history visualization and Retrieval-Augmented Generation (RAG)-enabled AI, the proposed system transforms previously scattered and isolated medical data into a unified, interconnected ecosystem. This integration establishes meaningful relationships between patients, healthcare providers, and medical records, promoting continuous, coordinated, and personalized healthcare delivery.

Purpose

The primary purpose of this project is to develop a unified healthcare platform that securely consolidates patients' medical records from multiple institutions. By leveraging technologies such as RAG-enabled AI and graphical data visualization, the project aims to enhance clinical insight and support medical decision-making. Furthermore, the platform seeks to empower patients to monitor their own health data while maintaining full control over information sharing through robust RBAC mechanisms. Ultimately, this initiative strives to ensure privacy, data integrity, and interoperability across the healthcare ecosystem to enhance patient trust and system reliability.

Objective

The key objectives of this project are as follows:

- To develop a unified healthcare platform that securely consolidates patients' medical records from multiple institutions.
- To empower patients to manage and monitor their own health data while maintaining full control over information sharing through RBAC mechanisms.
- To enable secure and efficient communication between patients, doctors, and caregivers.
- To provide data visualization tools, including a medical history graph, to help healthcare providers interpret patient data trends and clinical progress.
- To integrate RAG-enabled AI capabilities to generate automated reports and actionable insights that support doctors in diagnosis and medical decision-making.
- To ensure privacy, data integrity, and interoperability across the healthcare ecosystem to enhance patient trust and system reliability.

1.1 Digital Health Market Overview

The global push towards digital transformation in healthcare is driven by the need for efficiency, accessibility, and improved patient outcomes. According to Grand View Research, the global digital health market size was valued at USD 288.55 billion in 2024 and is projected to reach USD 946.04 billion by 2030, expanding at a Compound Annual Growth Rate (CAGR) of 22.2% from 2025 to 2030 [1].

This rapid growth is fueled by factors such as the increasing penetration of smartphones, advancements in internet connectivity, and government initiatives promoting digital health infrastructure [1]. However, realizing this potential requires overcoming the challenge of data silos. Currently, healthcare data is often locked within proprietary systems, making *interoperability*—the ability of different information systems to access, exchange, integrate, and cooperatively use data in a coordinated manner—a critical requirement for the modern healthcare ecosystem [3].

1.2 HL7 - Health Level Seven

Health Level Seven (HL7) International is a standards development organization that has defined the framework for the exchange, integration, sharing, and retrieval of electronic health information for decades [2]. Understanding the evolution of these standards is crucial to appreciating the architectural shifts in modern healthcare applications.

1.2.1 Evolution of Standards

- **HL7 v2 (The Workhorse):** Established in 1989, HL7 v2 remains the most widely implemented standard for hospital workflows (e.g., ADT - Admission, Discharge, Transfer messages). It utilizes a non-XML, pipe-delimited syntax (e.g., `MSH|^~\&|...`). While highly efficient for legacy backend systems, its lack of semantic strictness leads to significant implementation variances, posing challenges for modern web and mobile application development [2].
- **HL7 v3 (The Theoretical Model):** Introduced in the 1990s to address the vagueness of v2, HL7 v3 was built on a rigorous Reference Information Model (RIM). It enforced strict semantic correctness using complex XML structures. However, this theoretical purity resulted in a steep learning curve and bloated message sizes, leading to poor adoption rates and a lack of backward compatibility with v2 [2].
- **HL7 CDA (Clinical Document Architecture):** A component of v3, CDA focuses on the exchange of clinical documents (such as discharge summaries). While effective for static snapshots of care, it is document-centric rather than data-centric, making it difficult to extract granular data (like a single heart rate reading) required for real-time interactive applications [4].

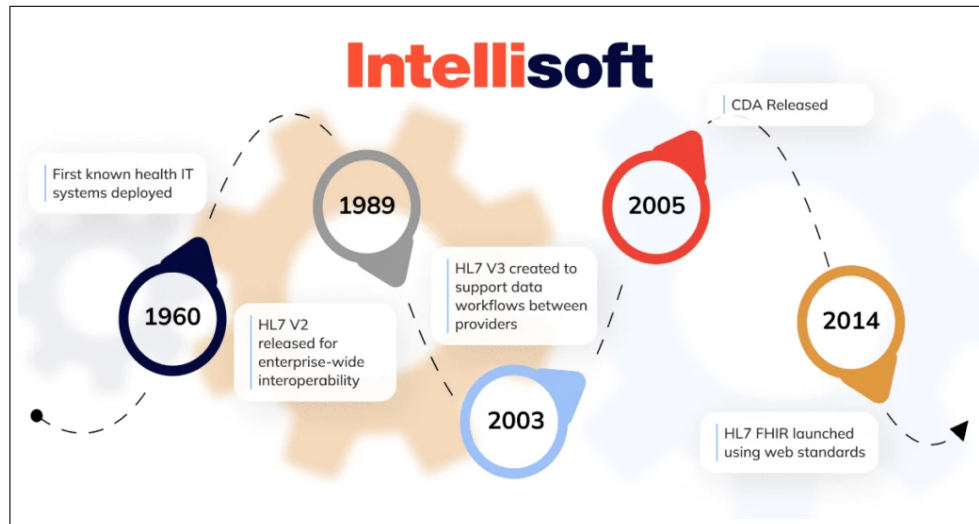


Figure 1.1: Evolution of HL7 Standards from Messaging to RESTful Resources.

1.3 Fast Healthcare Interoperability Resources (FHIR)

In response to the complexity of HL7 v3 and the limitations of v2, HL7 introduced **FHIR** (Fast Healthcare Interoperability Resources) in 2012. FHIR represents a paradigm shift from complex, custom healthcare protocols to modern web standards.

1.3.1 Core Architecture

Unlike its predecessors, FHIR is built on the RESTful API approach widely used in modern IT industries (e.g., by Google, Facebook, Twitter). It treats data elements as independent "Resources"—modular components like **Patient**, **Observation**, or **Medication**—that can be accessed and manipulated via standard HTTP methods (GET, POST, PUT, DELETE) [4].

1.3.2 Key Technical Advantages

- **Granularity via Resources:** FHIR allows systems to access specific data elements using unique URLs (e.g., `GET /Patient/123`), rather than exchanging entire heavy documents. This makes it ideal for mobile devices with limited bandwidth [4].
- **Modern Formats (JSON & XML):** While previous standards relied on complex XML or pipe-delimited text, FHIR natively supports JSON (JavaScript Object Notation), which is lightweight and directly parsable by web browsers and mobile apps [2].

- **The 80/20 Rule:** FHIR focuses on defining the 20% of requirements that satisfy 80% of common healthcare use cases. For the remaining complex or local requirements, it uses a built-in extension mechanism, ensuring the core standard remains simple and implementable [3].

1.3.3 Comparison of Standards

Table 1.1 highlights the fundamental differences between the major HL7 standards, illustrating why FHIR was chosen as the interoperability backbone for this project.

Table 1.1: Comparison of HL7 v2, v3, CDA, and FHIR Standards

Feature	HL7 v2	HL7 v3 / CDA	FHIR
Primary Paradigm	Event-Driven Messaging	Reference Model (RIM) / Documents	RESTful API & Resources
Data Format	Pipe-delimited text	Complex XML	JSON, XML, RDF
Transport	LLP / MLLP (TCP/IP)	SOAP / Web Services	HTTP / REST
Complexity	Moderate (Vague semantics)	High (Steep learning curve)	Low (Web-standard based)
Use Case	Hospital Backend (ADT, Orders)	Clinical Documents (Summaries)	Mobile, Web, Cloud, & Backend

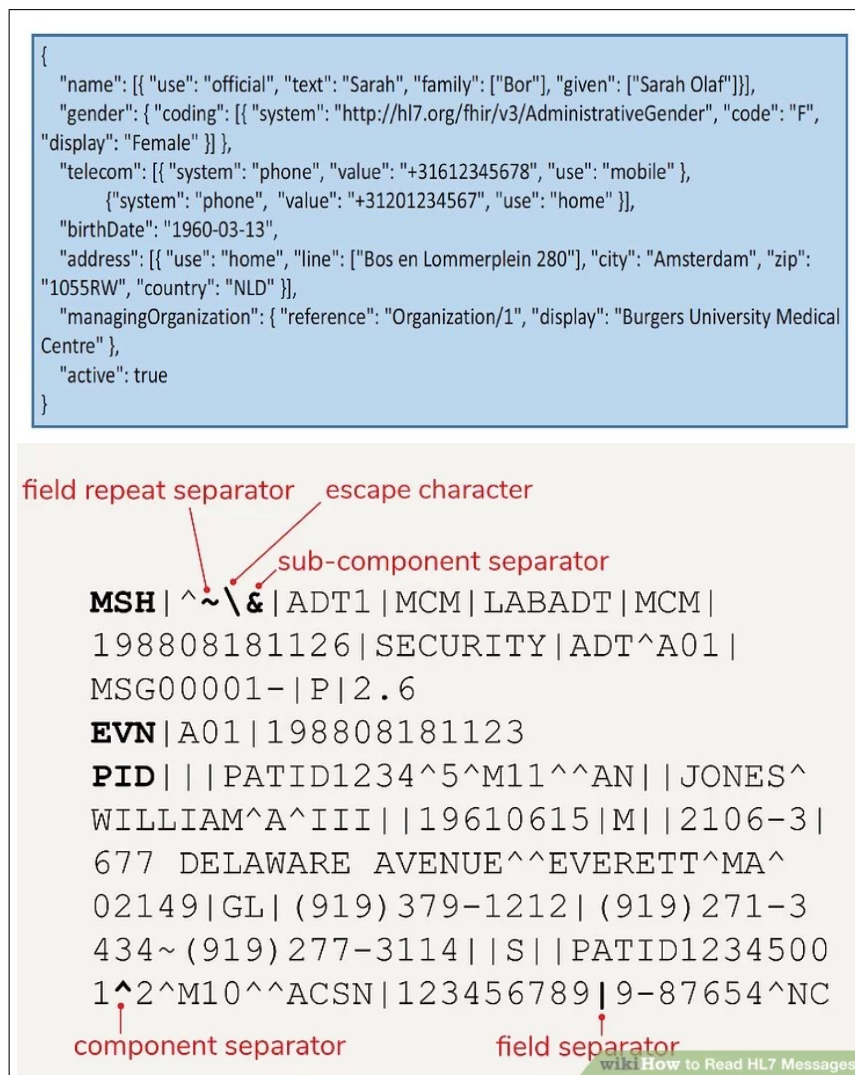


Figure 1.2: Visual Comparison of Data structure: HL7 v2 Message vs. FHIR JSON Resource.

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Conclusion and Future Work

Conclusion and future work go here.

Appendix

Appendix A

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